

FINAL CHALLENGE

Your NICU Rounds

Name: _____

Date: _____

How to play: For each question, put an X in the box next to the best answer (one answer per question). Get **9 of 12** right to earn the **stethoscope** badge. Take your time - real doctors think before they answer!

1. Bed 1: A baby born at 26 weeks keeps drifting cold even in a normal crib. Why can't this baby hold its own temperature?

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A. The baby doesn't try hard enough

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B. Thin skin, no insulating fat, no shivering, and almost no brown fat – plus lots of skin surface for such a tiny body

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C. The hospital is too big

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D. Babies are cold-blooded like lizards

2. Bed 2: The pulse oximeter on a 27-weeker getting extra oxygen reads a steady 100%. Your move?

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A. Perfect – leave everything as is

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B. Turn the oxygen UP to be safe

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C. Turn the extra oxygen DOWN a bit – sitting at 100% on oxygen risks the baby's eyes and lungs

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D. Remove the probe, it must be broken

3. Bed 3: The monitor alarms – flat breathing line for 20 seconds, heart rate dipping to 80. The nurse rubs the baby's back and everything recovers. What just happened?

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A. A nightmare

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B. A classic A-and-B spell: apnea of prematurity with bradycardia – the immature brain forgot to breathe

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C. The monitor was unplugged

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D. The baby was holding its breath on purpose

4. Bed 4: A 25-weeker's MAP is reading 22 (below the born-at-25-weeks rule of thumb). Before treating, what do you check FIRST?

- ☐ A. The baby itself: pee output, warmth, cap refill, blood tests — is blood actually being delivered?
- ☐ B. Whether the number is divisible by two
- ☐ C. The weather
- ☐ D. Nothing — always start dopamine immediately

5. Bed 5: A baby born at 29 weeks is grunting, flaring its nostrils, with skin sucking in between the ribs. Chest X-ray shows hazy lungs. Diagnosis and treatment?

- ☐ A. A cold — wait it out
- ☐ B. RDS from missing surfactant — support with CPAP, and give surfactant down a tube if needed
- ☐ C. Hiccups — apply juice
- ☐ D. Too much exercise

6. Bed 6: Why is the 26-weeker in bed 6 fed 2 mL of milk through a nose tube instead of a bottle?

- ☐ A. Bottles are sold out
- ☐ B. The suck-swallow-breathe rhythm isn't wired until about 32-34 weeks — bottle milk could end up in the lungs
- ☐ C. The baby prefers tubes
- ☐ D. Nurses find bottles boring

7. Bed 6, continued: The nurse notices this baby's belly is newly swollen and the last feed never emptied from the stomach. Why does everyone move fast?

- ☐ A. Swollen bellies are good luck
- ☐ B. These can be early signs of NEC, the dangerous gut disease of preemies — stop feeds, image the belly, act quickly
- ☐ C. The baby is just chubby
- ☐ D. It means the baby is ready for pizza

8. Bed 7: A 3-day-old term baby is noticeably yellow. The skin meter reads high, blood test confirms. What's the treatment?

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- A. Blue-light phototherapy (plus plenty of milk) – light twists bilirubin into flushable shapes
- B. A warm bath to wash the yellow off
- C. Waiting a month
- D. Yellow paint to match

9. Bed 8: A baby was born last night at 27 weeks. Nothing seems wrong neurologically. Does this baby still need a head ultrasound this week?

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- A. No – no symptoms, no scan
- B. Yes – IVH brain bleeds in very early preemies usually happen in the first days and are often silent; screening through the fontanelle is routine
- C. Only if the parents insist
- D. Head ultrasounds are for adults

10. Bed 9: The echo on a tiny preemie shows the ductus arteriosus still wide open (a PDA), flooding the lungs. Which is NOT one of your real options?

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- A. Watch and wait – many close on their own
- B. A medicine like ibuprofen to nudge the vessel shut
- C. A tiny plug threaded in through a leg vein
- D. Asking the baby to concentrate very hard on closing it

11. Rounds pop-quiz from your attending: Why does the NICU target 90-95% oxygen saturation for preemies instead of 100%?

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- A. The machines get tired above 95
- B. Because huge studies showed too much oxygen damages eyes and lungs, while too little costs lives – 90-95% balances both dangers
- C. Insurance rules
- D. Round numbers are unlucky

12. Last stop: A mother is doing kangaroo care, her 30-weeker asleep on her chest, monitor numbers rock-steady. A visitor asks why 'just cuddling' is allowed in intensive care. Your best answer?

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A. It's not allowed – sound the alarm

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B. Skin-to-skin contact is real treatment: it holds the baby's temperature, steadies heart rate and breathing, and supports feeding and bonding

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C. The nurses needed a break

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D. It's for photos only

Score: _____ / 12

Badge earned? YES / NOT YET (need 9)